# Expert Brief — request for an independent cardiology opinion

Consumer case DC/44/CC/423/2026 (District Consumer Commission, Chandigarh): heirs of Late Sh. Manohar v. PGIMER. Prepared on 30.10.2026 from the court record. Please give your own, independent view on the documents.

**Contents:** 1. Patient and question · 2. Timeline · 3. Documents · 4. Questions · 5. Literature · 6. Request

## 1. The patient and the question

A 60-year-old man with sputum-positive pulmonary TB on treatment, newly diagnosed type 2 diabetes, earlier drug-induced hepatitis, low sodium and fluid around both lungs was referred from GMSH-16 to PGIMER on 11.03.2026 for ECG changes read as Wellens. PGIMER recorded "CAD – USA (Wellen's pattern)" and anaemia (Hb 7.5 g/dL), planned to "correct anaemia", and treated him for acute coronary syndrome in its CCU. A cross-matched blood unit was never issued. On 12.03.2026 he was moved out of the CCU onto a trolley for want of a ward bed. He was seen once at about 9:00 pm; the next record is at 12:15 am on 13.03.2026, when he was received in the CCU unresponsive and pulseless; he was declared dead at 12:51 am. **The question:** was the care consistent with accepted practice in March 2026, and if not, did any shortfall cause or contribute to the death?

## 2. Short timeline

| Date, time | Event (source) |
|---|---|
| 10.03, 1:07 pm | Hb 8.2 g/dL at GMSH-16 (C-7, p.47); troponin T negative (C-6) |
| 11.03, 12:14 am | Referral "regarding his recent ECG findings [CAD ?ACS?] (Wellens) (loading dose given)" (C-6, p.46) |
| 11.03 (emergency) | BP 130/100, pulse 116, SpO2 94%; "Imp - Wellens Syndrome" (C-8, p.52) |
| 11.03, 10:10 am | CCU; chest pain from 11 pm on 10.03; ECG rate 123, new T changes V2–V4; echo "(N) LVEF"; "CAD – USA (Wellen's pattern)"; plan "O2, correct anaemia, CXR"; Hb 7.5 (OP Ann-2, 3, 6) |
| 11.03 | Point-of-care: TNI <0.05, CKMB <1.0, BNP 37.4, myoglobin 176 (range 0–107), D-dimer 3280 (range 0–500) (OP Ann-1, undated) |
| 11–12.03 | Drugs: ticagrelor, aspirin, enoxaparin, nitrate, furosemide 40 mg then 20 mg IV twice daily, IV fluid 30 ml/hr, rifampicin and other TB drugs (OP Ann-8) |
| 11.03–12.03 chart | Pulse 128/125/126 (11.03); BP 90/49 (6 pm); 87/46 at 9 am on 12.03 (pulse "45", unclear); 110/73 at 1 pm; ~93/60, pulse 114 at 3 pm; 90/60, pulse 82 at 9 pm; gaps of 4, 8, 4 and 6 h (OP Ann-5) |
| 12.03 | Blood requested 9:15 am; cross-matched 12:55 pm; never issued (OP Ann-7; C-10) |

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|---|---|
| 12.03, ~5:10 pm | Out of the CCU onto a trolley (admitted; time inferred from "nearly 31 hours") |
| 12.03, ~9:00 pm | Brought to the CCU: "mild breathlessness" (hospital) or chest pain (family); pulse and saturation "within normal limits"; auscultation "similar to when he was admitted"; not re-admitted; no ECG on record (WS paras 12, 14) |
| 13.03, 12:15 am | "unresponsive & pulse was not palpable"; CPR; atropine and adrenaline; "ECG done" at 12:51; declared dead (OP Ann-5) |
| Certificate | I (a) Type I respiratory failure, (b) aspiration pneumonia, (c) coronary artery disease; II pulmonary TB; no autopsy (C-11) |

## 3. Documents to read

Complaint bundle (court pages): C-1 to C-5 (pp.36–45), C-6 (p.46), C-7 (pp.47–50, incl. two ECGs), C-8 (pp.51–54), C-9 (p.55), C-10 (pp.56–58), C-11 (index p.59). PGIMER's written statement (WS pp.1–12, especially paras 3, 8, 10–15, 17–20) and its annexures 1–12 (pp.13–25), especially Ann-1, 2, 3, 5, 6, 7, 8, 11 and 12.

## 4. Questions (please answer each separately)

1. On the records of 11–12 March, what level of care (CCU or ICU) and what frequency of observation was expected?
2. How far does a negative troponin change the risk carried by a Wellens pattern?
3. With Hb 7.5 g/dL in a patient treated for ACS (with TB, and on ticagrelor, aspirin and enoxaparin), was transfusion indicated, and how urgently? What did the guidance in force in March 2026 say, and what does "correct anaemia" mean here? Was a repeat Hb expected on 12.03?
4. Angiography was "deferred" (WS para 20-a). What reasons justify deferral with this pattern, and where and how closely should such a patient be watched? If anaemia was a reason, what was the place of transfusion?
5. Was it reasonable to move him out of the CCU at about 5:10 pm (after pulse 114 and BP ~93/60 at 3 pm), and to send him back to the trolley after the 9:00 pm check? What assessment and hand-over were expected?
6. Could "mild breathlessness" in a diabetic patient under ACS treatment be an anginal equivalent? Were an ECG, troponin or re-admission expected at 9:00 pm?
7. Do the chart values (pulse 125–128; BP 90/49, 87/46, 90/60; respiration 24–26 on 11.03) show instability needing escalation? What frequency of observation would you expect (the drug chart's "vitals q_h" is unreadable)?
8. Had he been observed between 9:00 pm and 12:15 am, could the arrest have been detected earlier, and with what likely difference?
9. Does the certified cause (aspiration pneumonia with coronary artery disease; TB) fit the course? Is fluid in the trachea at intubation during CPR a cause or a result of the arrest?

10. Can the share of the earlier illnesses be separated from any shortfall in care?
11. What do furosemide 40 mg then 20 mg IV twice daily and IV fluid 30 ml/hr say about his state? Can BNP 37.4 be compared with NT-proBNP 1117.48?
12. What do OP Ann-2 (rate 123; new T changes V2–V4), Ann-11/12 and the GMSH-16 ECGs show?
13. Did the flagged D-dimer (3280) and myoglobin (176) call for further work-up, or are they explained by TB, effusion or infection?
14. Rifampicin and ticagrelor were given together (OP Ann-8). Is this interaction relevant to his treatment or outcome?

## 5. Literature you may wish to consider

For your independent view only; please say whether each applies to this patient.

- AABB International Guidelines for red blood cell transfusion, 2023 (JAMA; PubMed 37824153).
- ACC/AHA 2025 guideline for acute coronary syndromes — transfusion recommendation (reported in AABB News, 12.03.2025).
- AABB 2025 guideline on transfusion in acute myocardial infarction (ACC Journal Scan, 27.08.2025).
- StatPearls, "Wellens Syndrome" (NCBI Bookshelf NBK482490).

## 6. What we request

- A written opinion with reasons, answering each question separately.
- For each point, keep apart: (a) whether care fell below the expected standard, (b) what harm followed, and (c) whether that shortfall caused or contributed to the death.
- Your qualifications and a statement of any connection with PGIMER or the parties.
- Any further record you need to answer (we will ask the advocate to seek it).